June 13, 2025

Robert F. Kennedy, Jr.
Centers for Medicare & Medicaid Services
Department of Health and Human Services
Attention: CMS–4211–PN
P.O. Box 8016
Baltimore, MD 21244–8016.

Re: Request for Information; Health Technology Ecosystem

Dear Mr. Kennedy,

Thank you for the opportunity to provide information to HHS regarding the Health Technology Ecosystem. Denver Health is a comprehensive, integrated organization that delivers preventative, primary, specialty, and acute care services. This integration promotes continuity of care for each patient and assures that health care is delivered in the most efficient, cost-effective manner. We care for 33% of Denver's population annually and one in three Denver-area children. As Colorado's primary safety-net institution, Denver Health has provided billions of dollars in uncompensated care and serves as a model for other safety net institutions across the nation. Below are our answers to the questions asked as part of the RFI.

C. Providers

This section is intended for all stakeholders to provide input on questions as they relate to use cases and workflows that involve providers. While we certainly want providers to answer questions in this section (and in other sections) from the provider point of view, we also invite all stakeholders to provide their viewpoints on the provider workflows as appropriate.

1. Digital Health Apps

PR-1. What can CMS and its partners do to encourage providers, including those in rural areas, to leverage approved (see description in PC-5) digital health products for their patients?

a. ***What are the current obstacles?*** State agencies and their contracted service providers are not always aware of the opportunities to leverage these new capabilities and gravitate to older interfaces and data sharing methods. Often, this leads to fragmentation or complete lack of interoperability between different digital products,

requiring providers to download information, manually transfer it to another platform, and re-enter it.

PR-2. What are obstacles that prevent development, deployment, or effective utilization of the most useful and innovative applications for physician workflows, such as quality measurement reporting, clinical documentation, and billing tasks? How could these obstacles be mitigated? The biggest obstacles are funding issues and lack of knowledge on how useful these applications can be. The funding issues are most often related to the actual purchasing and implementation of the technology—with organizations focusing limited financial resources on care provision rather than technology upgrades--but in many cases the lack of funding to support the technology is also a long-term problem with keeping up with costs of maintenance. Providing financial incentives may help mitigate some of these issues.

PR-3. How important is it for healthcare delivery and interoperability in urban and rural areas that all data in an EHR system be accessible for exchange, regardless of storage format (for example, scanned documents, faxed records, lab results, free text notes, structured data fields)? This is very important. At Denver Health, we treat many patients from both urban and rural areas in Colorado, Wyoming, and other surrounding states. It is imperative for patient care that all data be accessible for exchange between healthcare organizations. Interoperability allows us to get the data we need quickly, so we can provide the best care for the patient without un-necessary delays or the need to repeat expensive diagnostic testing that has already been performed elsewhere but for which data is not visible.

PR-4. What changes or improvements to standards or policies might be needed for patients' third-party digital products to have access to administrative workflows, such as auto-populating intake forms, viewing provider information and schedules, and making and modifying an appointment? Many of these examples are already possible in our provider portal. CMS adding certification criteria is one way of ensuring that EHR developers create the requested functionality.

2. Data Exchange

PR-6. Is TEFCA currently helping to advance provider access to health information?

a. ***Please provide specific examples.*** TEFCA provides healthcare organizations and providers with a single on-ramp to participate in national interoperability. Provider

organizations no longer need to connect to multiple exchanges; rather, when they connect to TEFCA they are able to get data from every QHIN. Querying data versus pushing data (via direct messaging) is more valuable to providers as they are able to pull the data they need, when they need it.

b. ***What changes would you suggest?*** More incentives for people to join TEFCA, and eventually penalties for not joining, would spur better participation. We would also like to see other purposes of use and their related SOP's rolled out in a timely manner. These purposes of use should first be optional, then required, over time.

c. ***What other options are available outside of TEFCA?*** Other national frameworks such as eHealth Exchange, Carequality, and Commonwell are useful, but not as helpful as TEFCA, as all of these alternatives are stand-alone exchanges. As a result, an organization must connect to each one individually to exchange data with participants. There are also regional HIE's which are an older, antiquated solution that depends on point-to-point interfaces to transmit data. Regional HIEs are no longer as beneficial as nation-wide exchanges, yet many regional HIEs continue to press policymakers for financial or regulatory support to sustain their systems, despite the fact that these are rapidly becoming obsolete.

d. ***Are there redundant standards, protocols or channels or both that could be consolidated?*** We believe as more organizations move onto TEFCA, exchanges that do not become QHINs will no longer be needed and will likely find a new purpose. There will be a need for push (direct address) workflows for certain use cases, even though pulling (querying) is usually more beneficial.

PR-7. What strategies can CMS implement to support providers in making high-quality, timely, and comprehensive healthcare data available for interoperability in the digital product ecosystem? Incentivize or penalize providers to install certified EHR software and connect to TEFCA.

How can the burden of increasing data availability and sharing be mitigated for providers? Incentive programs could help cover the cost of the software and technical support needed.

PR-8. What are ways CMS or partners can help with simplifying clinical quality data responsibilities of providers? Align programs, including the tools used to collect and report data, the actual measures within the programs (when possible), and the timelines and reporting periods.

a. ***What would be the benefits and downsides of using Bulk FHIR data exports from EHRs to CMS to simplify clinical quality data submissions? Can CMS reduce the burden on providers by performing quality metrics calculations leveraging Bulk FHIR data exports?*** If CMS allowed for one bulk FHIR submission to submit data for every program (IQR, MIPS, PI, OQR, etc.), this would help reduce the reporting burden. Currently, each program requires unique effort. However, if each program continues to require data to be submitted individually, we see no significant benefit for provider organizations using Bulk FHIR data exports from EHR's to submit quality data. The current process for quality data submission for most programs is not overly burdensome.
b. ***In what ways can the interoperability and quality reporting responsibilities of providers be consolidated so investments can be dually purposed?*** CMS could look into eventually plugging into the TEFCA or FHIR API framework to query data from organizations for quality measures. Interoperability is really moving from a "push" to "pull" methodology, which may not be ideal for quality reporting.

3. Digital Identity

PR-9. How might CMS encourage providers to accept digital identity credentials (for example, CLEAR, ID.me, Login.gov) from patients and their partners instead of proprietary logins that need to be tracked for each provider relationship? CMS should first add this requirement as a certification criterion, so software developers make the needed updates to support this functionality. Then, adding this requirement to a program such as MIPS or PI would encourage providers to accept these credentials.

a. ***What would providers need help with to accelerate the transition to a single set of trusted digital identity credentials for the patient to keep track of, instead of one for each provider?*** We would look to the EHR vendors to provide feedback on whether this is something that they could support via patient portals. One request for HHS is to consider how this will align with information blocking regulations, mainly in cases where the patient cannot perform the digital identity credentials needed to access their EHR data. Should hospitals and provider organizations have an alternative way for patients to access their portal outside of these digital identity credentials? Healthcare organizations would need to be assured that we would not be accused of information blocking.

PR-10. Regarding digital identity credentials (for example, CLEAR, Login.gov, ID.me, other NIST 800-63-3 IAL2/AAL2 CSPs):

a. ***What are the challenges and benefits for providers?*** We would save time and effort by not having to help patients log in, perform password resets, etc. On the other hand, we may lose the ability to help our patients gain access to their health information if identifying information lives exclusively with patients and not within care delivery systems, as it does today. Patients may call our help desk hoping to get support, and we may not be able to help them like we can currently.
b. ***How would requiring their use improve access to health information?*** It would prevent patients from having to remember multiple usernames and passwords, which may allow them to access their information more quickly and easily. However, it is important to note that identity verification is just one of three items needed for us to release data. We also need to perform patient matching and receive patient authorization under existing regulations.
c. ***What are the potential downsides?*** It is unclear what role hospitals and provider organizations would have when patients have issues with digital identity verification. Would we be able to assist when they have issues, or would we have to refer patients to an outside vendor for support accessing their EHR data?
d. ***What impact would mandatory credentials have on a nationwide provider directory?*** While there may be some overlap, we would not expect there to be much of an impact.
e. ***How could digital identity implementation improve provider data flow?*** We would not expect that digital identity implementation would improve provider data flow.
f. ***Would combining FHIR addresses and identity improve data flow?*** No, and we would not recommend trying to combine FHIR addresses and identity. Identity is tied to an individual person, regardless of any organizational affiliations. FHIR addresses are organization specific.

PR-11. How could members of trust communities (for example, QHINs, participants and subparticipants in TEFCA, which requires Identity Assurance Level 2 (IAL2) via Credential Service Providers (CSPs)) better support the goals of reduced provider and patient burden while also enhancing identity management and security? This could be accomplished by ensuring that QHINs are performing thorough vetting of participants and subparticipants, while also ensuring the queries performed align with the SOP for that purpose of use. It will also be

vital to allow for objections from QHIN governance councils and create and support an escalation path when adjudication is needed. Transparency and trust are key to making TEFCA successful.

On behalf of Denver Health, we appreciate the opportunity to provide comments. Thank you for your consideration.

Sincerely,

Read G. Pierce, MD

Chief Medical Officer